

Supporting grief, one visit at a time

A practical tip sheet for community-based MHPSS workers after the Nepal floods

Offer care, not a timetable for recovery. Support the person’s own choices, community connections and practical needs.

BEFORE YOU VISIT Know the local risks, your supervisor and the verified health, emergency and Child Protection pathways. Arrange language and accessibility support. Work within your training; this sheet is not a diagnostic or treatment tool.

01 **LOOK · Start with safety and needs**

Introduce yourself and ask permission to talk in a safer, private place. Check health, protection, basic needs and care for dependants. Immediate danger? Act now—see the urgent box below. Notice who needs extra support; do not judge grief by tears or appearance.

“What would help you most today?”

02 **LISTEN · Make room, without pressure**

Let the person speak, stay quiet or stop. Acknowledge their loss without comparing experiences or promising recovery. People grieve differently. Do not ask for a detailed retelling, say “be strong”, or insist they move on.

“We can talk—or sit quietly.”

03 **LINK · Support their way of mourning**

Ask what family, community, faith or non-religious support they want. If they wish, ask about mourning practices and obstacles. Help connect them with their chosen people and practical assistance. Never prescribe rites, touch, prayer or disclosure.

“Is there something important you have not been able to do?”

04 **CONNECT · Offer more help when needed**

Seek supervisor or health-worker support if distress worsens, persists or disrupts daily life. Help the person reach agreed care. Link separated or unaccompanied children, or children at risk, to Child Protection. Do not diagnose or provide grief therapy beyond your role.

Do not wait six months to offer support or refer.

05 **FOLLOW UP · Agree one next step**

Ask what feels manageable and agree when and how to reconnect. Check whether practical help or a referral was reached. Record only the minimum required in the existing confidential system; share only as authorised. Use supervision and case review.

“May I check in again? What would work for you?”



WHEN SOMEONE IS MISSING

Hope and grief can coexist. Do not assume death or push for “closure”. Respect choosing to wait or to hold rites. Link families to verified tracing or identification information; never promise answers or a date.



URGENT CONCERN? ACT NOW.

If there is immediate danger, self-harm or suicide risk, severe confusion, or inability to meet essential care needs: seek urgent health or emergency help and alert your supervisor. If safe, stay with the person or arrange a trusted support person until help takes over. Follow your trained protocol; do not put yourself at risk.

SAMPLE · ENGLISH REVIEW DRAFT · v1 · NOT FOR FIELD DISTRIBUTION

Based on the project Grief Support Framework (revised 7 Oct 2026), WHO Psychological First Aid (2011) and WHO/UNHCR mhGAP-HIG, GRI (2015). Follow-up is an added programme step, not a fourth PFA action principle.